

East Bay Family Medicine

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Patient: David Park
Date of Birth: 09/24/1986
Patient ID: 7782041
Date of Encounter: June 3, 2024
Encounter Type: Office Visit, Established Patient
Provider: Marcus Hayes, MD

SUBJECTIVE

Mr. Park returns to clinic 3 weeks after acute onset of low back pain. He was initially seen at Sutter Urgent Care on 05/12 (records reviewed) and treated conservatively with NSAIDs and cyclobenzaprine. He reports the pain has improved somewhat but has not resolved. He estimates about 30-40% improvement from peak intensity.

Pain is now characterized as a constant ache in the right low back, 4/10 at rest, 6/10 with prolonged sitting (his work involves 8+ hours at a desk daily). He has noted intermittent radiation into the right buttock over the past week, and twice in the past three days has felt a brief "electric" sensation down the right posterior thigh, lasting seconds. No frank weakness. No numbness. No bowel or bladder symptoms.

He completed the 5-day course of cyclobenzaprine. He continues ibuprofen as needed but is concerned about taking it daily. He has been using a heating pad at his desk and has shifted to working from a standing desk for portions of the day.

OBJECTIVE

Vitals: BP 124/80, HR 72, weight 178 lb (stable).

Examination of the back reveals mild paraspinal tenderness on the right at approximately the L4-L5 level. The lumbar lordosis appears preserved. Range of motion is mildly limited in forward flexion. Straight-leg raise on the right reproduces the patient's familiar back pain at approximately 60 degrees, with a mild pulling sensation but no clear radicular symptoms during the maneuver. Straight-leg raise on the left is negative. Strength is 5/5 throughout both lower extremities. Deep tendon reflexes are 2+ and symmetric. Sensation is intact to light touch in all dermatomes bilaterally.

ASSESSMENT

1. Subacute low back pain with mild radicular features, right L5 distribution most likely (M54.16). The picture has evolved from a pure mechanical strain to include occasional radicular symptoms, raising suspicion for early disc involvement. The exam is reassuring — no motor deficit, intact reflexes — but the new radiating symptoms warrant escalation.

PLAN

1. Referral to orthopedic spine specialist (Dr. Reza Tehrani at Bay Area Orthopedic Specialists). Will request expedited scheduling given symptom evolution.

2. Continue NSAIDs as needed. Discussed switching to naproxen 500 mg BID for more sustained coverage; patient agreeable.
3. Encouraged ergonomic evaluation of his work setup. He will request a standing desk assessment from his employer's HR.
4. Will defer imaging at this point pending orthopedic evaluation.
5. Return to clinic if symptoms worsen, if any motor weakness develops, or if bowel/bladder symptoms appear (counseled to seek emergency care for the latter).

Counseled extensively on activity modification and the typical course of disc-related back pain. Approximately 20 minutes spent on counseling.

— *Marcus Hayes, MD*